The strongest clearly established right you can invoke is that significant liberty restrictions, including parole/supervision conditions and mandatory treatment, may not be imposed or continued without individualized justification and procedural safeguards, and may not be prolonged indefinitely based solely on categorical status or blanket policy.[1][2][3][4]

Below I've organized the key Supreme Court and Ninth Circuit authorities, how close they are factually to your scenario, and how they can be framed at the right level of specificity for the qualified-immunity prong.

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Supreme Court: Parole and Continued Confinement

Morrissey v. Brewer, 408 U.S. 471 (1972)

- **Citation and holding:** 408 U.S. 471 (1972). The Court held that parole revocation deprives a parolee of a protected liberty interest and therefore requires a two-stage process with notice, opportunity to be heard, and an evidentiary hearing before a neutral decisionmaker.[5][1]
- **Factual context:** Iowa parolees had their parole revoked without any hearing; the Court required procedures to ensure revocation is based on "verified facts" and a reasoned judgment that revocation is warranted.[1][5]
- **Relevance/analogy:** Morrissey establishes that the *continuation or termination* of conditional liberty (parole) requires individualized fact-finding and cannot be handled by automatic or purely administrative shortcuts. Your policy functionally extended a condition of supervised release (mandatory treatment) to the maximum term without individualized consideration of progress or risk—an analogous deprivation of conditional liberty without the sort of individualized process Morrissey demands.[1]
- **Precedential status:** Binding Supreme Court authority; repeatedly treated as the baseline for due process in parole and supervised-release contexts.[6][5]
- **Procedural vs substantive:** Purely **procedural due process**; the case is about what process is required when the State alters the nature or duration of conditional liberty, not about a substantive right to parole.

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Supreme Court: Nature and Duration of Confinement

Foucha v. Louisiana, 504 U.S. 71 (1992)

- **Citation and holding:** 504 U.S. 71 (1992). The Court held that the State may not *indefinitely* confine an insanity acquittee based on dangerousness alone once the original medical justification (mental illness) no longer exists; commitment must rest on continuing, individualized justification that links the nature and duration of confinement to both mental illness and dangerousness.[2][7][8]
- **Factual context:** Foucha was found not guilty by reason of insanity and committed; later evidence showed he was no longer mentally ill, though the State asserted he remained dangerous. Louisiana law allowed continued confinement based solely on dangerousness. The Court held that this violated due process because the ongoing restriction of liberty no longer had a valid, individualized medical basis.[7][8][2]
- **Relevance/analogy:** Foucha provides the principle that continued confinement or comparable liberty restrictions cannot be justified solely by a prior status (insanity acquittee, sex-offender parolee) or generic dangerousness; the State must periodically assess whether the original justification (mental illness; in your case, treatment need/risk) still exists and still supports the restriction. Your blanket policy categorically denied completion and thereby prevented any determination that treatment was no longer necessary, effectively allowing indefinite continuation of a liberty restriction based on status alone.[8][2]
- **Precedential status:** Binding Supreme Court authority; regularly cited for the rule that "the nature and duration of commitment must bear some reasonable relation to the purpose for which the individual is committed."[8]
- **Procedural vs substantive:** Foucha is often framed as **substantive due process**—the State cannot hold someone when the substantive justification no longer exists—but it also has a procedural component: the requirement of periodic, meaningful hearings that reassess current mental illness and dangerousness. For QI, you can rely on its clearly established requirement of ongoing, individualized justification for continued confinement-like restraints.[7][8]

Vitek v. Jones, 445 U.S. 480 (1980)

- **Citation and holding:** 445 U.S. 480 (1980). The Court held that involuntary transfer of a prisoner to a mental hospital—a change that "stigmatizes" and qualitatively alters the nature of confinement—implicates a liberty interest and requires robust procedural safeguards: written notice, an adversary hearing, an independent decisionmaker, opportunity to present and confront witnesses, and a written statement of reasons.[9][10][11]
- **Factual context:** Nebraska allowed transfer of prisoners to a mental hospital based solely on a physician's finding of mental illness and treatment need, without any formal hearing. Jones was

transferred under this scheme and challenged the lack of process.[10][9]
- **Relevance/analogy:** Vitek underscores that when the State imposes or significantly alters a condition of confinement or supervision on the basis of medical judgments—here, psychiatric treatment—the decision must be individualized and accompanied by meaningful process. Your defendant approved a corporate treatment policy that *never* allowed individualized determination of completion before the full parole term; that is, he ensured that the "nature and duration" of the treatment condition could not be adjusted based on an individual's progress or current risk, contrary to the individualized decision-making Vitek requires.[9][10]
- **Precedential status:** Binding Supreme Court authority.
- **Procedural vs substantive:** Primarily **procedural due process**; the Court accepts the State's substantive power to transfer genuinely mentally ill inmates but insists on individualized, adjudicative procedures before imposing the more restrictive, stigmatizing condition.[10][9]

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Ninth Circuit: Sex-Offender Treatment and Classification

Neal v. Shimoda, 131 F.3d 818 (9th Cir. 1997)

- **Citation and holding:** 131 F.3d 818 (9th Cir. 1997). The Ninth Circuit held that (1) labeling an inmate a "sex offender" and compelling completion of a sex-offender treatment program as a precondition for parole eligibility implicates a liberty interest due to both stigma and its effect on parole; and (2) due process requires an individualized hearing with notice of reasons, opportunity to present evidence and witnesses, and to contest the classification.[4]
- **Factual context:** Hawaii's Sex Offender Treatment Program labeled inmates "sex offenders" based on prior conduct and compelled participation in a psychoeducational treatment program as a prerequisite to parole eligibility. Neal had never been convicted of a sex offense and had no prior opportunity to challenge the "sex offender" label or mandatory program.[4]
- **Relevance/analogy:** Neal is extremely close to your fact pattern. It addresses a mandatory sex-offender treatment program whose completion is effectively a condition of release; it holds that attaching this condition without individualized due process violates procedural due process. Your case involves a CDCR-contracted program that—per your allegations—never found anyone to have "completed" treatment and forced participation throughout parole based on blanket policy rather than case-by-case assessment. Neal gives you Ninth Circuit precedent that:[4]
  - Sex-offender treatment tied to parole eligibility is a significant liberty interest.
  - Due process requires individualized assessment and a hearing before imposing such treatment conditions.
  - A system that effectively coerces treatment via parole consequences without meaningful process

is unconstitutional.
- **Precedential status:** Binding Ninth Circuit panel decision; still cited by other circuits (e.g., Fifth Circuit) as clearly establishing a liberty interest in avoiding sex-offender classification and mandatory treatment tied to parole.[12]
- **Procedural vs substantive:** **Procedural due process**; the holding focuses on notice and hearing requirements before classification and treatment conditions that alter parole eligibility.[4]

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Ninth Circuit: Conditions of Supervised Release and Treatment

United States v. Williams, 356 F.3d 1045 (9th Cir. 2004)

- **Citation and holding:** 356 F.3d 1045 (9th Cir. 2004). The Ninth Circuit held that imposing a supervised-release condition requiring a defendant to take antipsychotic medication is an "especially grave infringement of liberty" and is permissible only if the district court makes (1) "on-the-record, medically grounded findings" that medication is necessary to accomplish one or more purposes in 18 U.S.C. § 3583(d)(1); and (2) an explicit finding that the condition "involves no greater deprivation of liberty than is reasonably necessary" under § 3583(d)(2).[3][13]
- **Factual context:** A federal defendant's supervised release included a condition that he take any prescribed psychotropic medication, with noncompliance potentially triggering revocation.[3]
- **Relevance/analogy:** Williams articulates a clearly established rule in the Ninth Circuit that **conditions of supervised release which significantly burden bodily integrity or liberty must be supported by individualized findings tailored to the defendant's characteristics and the purposes of supervision.** Your blanket policy ensured that every sex-offender parolee remained in intensive treatment throughout parole without any individualized judicial or clinical findings regarding necessity or proportionality, contrary to Williams's requirement that such conditions involve "no greater deprivation of liberty than is reasonably necessary."[3]

United States v. Weber, 451 F.3d 552 (9th Cir. 2006)

- **Citation and holding:** 451 F.3d 552 (9th Cir. 2006). The Ninth Circuit held that penile plethysmography as a supervised-release condition requires individualized determination under § 3583(d): the court must find the condition "reasonably related" to the offense and the defendant's history and that it "involves no greater deprivation of liberty than is reasonably necessary."[14][15]
- **Factual context:** A sex-offender's supervised-release conditions included mandatory penile plethysmography testing within a treatment program; the court vacated the condition because the district court failed to make individualized findings.[15][14]

- **Relevance/analogy:** Weber extends Williams's individualized-findings requirement to invasive sex-offender treatment/testing and stresses that whether such a condition will promote supervisory goals "must be an individualized determination." A corporate policy that *categorically* refuses ever to recognize completion, regardless of an individual's progress or risk, violates this tailoring requirement more clearly than Weber's own facts.[15]

United States v. Cope, 527 F.3d 944 (9th Cir. 2008)

- **Citation and holding:** 527 F.3d 944 (9th Cir. 2008). Cope reads Williams broadly: specialized findings are required for "any imposed treatment or medication that implicates a particularly significant liberty interest," not just antipsychotics.[13][14]
- **Factual context:** A defendant challenged special supervised-release conditions including psychiatric medication and sex-offender treatment and testing; the Ninth Circuit held that each condition affecting "special liberty interests" requires separate justification and individualized findings.[13][14]
- **Relevance/analogy:** Cope helps you argue that **mandatory participation in a sex-offender treatment program over the entire parole term is a condition implicating a "special liberty interest,"** and therefore, under clearly established Ninth Circuit law, cannot be imposed or extended via blanket policy without individualized findings that it is necessary and proportionate.[14][13]

United States v. Johnson, 2012 (Ninth Cir. published opinion)

- **Citation and holding:** United States v. Johnson, 11-30256 (9th Cir. 2012) (published), available at 2012 WL 5279672. The court upheld a condition requiring a sex-offender assessment, but emphasized that under § 3583(d) such conditions must involve "no greater deprivation of liberty than is reasonably necessary" and must be reasonably related to the offense and defendant's characteristics.[16]
- **Factual context:** Johnson's supervised release was modified to require a sexual-offender assessment because of prior sexual convictions and inability to confirm completion of sex-offender treatment.[16]
- **Relevance/analogy:** Johnson shows the Ninth Circuit applying the Williams/Weber framework directly in the sex-offender context: assessment and treatment conditions must rest on an individualized record and be proportionate. A policy that *never* allows an assessment to result in completion—effectively foreclosing proportionality—conflicts with this clearly established requirement.[16]

United States v. Tenorio, 769 F. App'x 492 (9th Cir. 2019) (reinforcing Cope)

- **Citation and holding:** 769 F. App’x 492 (9th Cir. 2019). Tenorio reaffirmed that Williams and Cope require special findings for “any imposed treatment or medication that implicates a particularly significant liberty interest,” citing Cope and Williams.[13]
- **Factual context:** The Ninth Circuit reviewed supervised-release conditions including treatment; it reiterated the requirement of individualized, on-the-record findings.[13]
- **Relevance/analogy:** Tenorio is useful for the time frame: by 2019—the beginning of your defendant’s alleged conduct—the individualized-assessment rule for liberty-infringing treatment conditions was firmly established in Ninth Circuit precedent.[13]

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Federal Law: Statutory Framework and Consensus on Individualized Assessment

Federal Supervised-Release Statute and Sentencing Commission Primer

- **Statutory rule:** 18 U.S.C. § 3583(d) permits discretionary conditions of supervised release only if they are “reasonably related” to sentencing factors and “involve no greater deprivation of liberty than is reasonably necessary” for deterrence, protection of the public, and rehabilitation.[15][3]
- **Sentencing Commission primer:** The U.S. Sentencing Commission’s 2026 supervised-release primer synthesizes appellate case law and emphasizes that courts must conduct an “individualized assessment” when imposing discretionary and special conditions—including sex-offender treatment and intrusive testing—and ensure conditions are tailored and proportionate.[17]
- **Relevance/analogy:** This statutory and guideline framework, as interpreted in Williams, Weber, Cope, Johnson, and cases from other circuits, bolsters your argument that by 2019–2025 there was a robust consensus that **conditions equivalent to continued mandatory treatment during supervised release require individual, case-specific justification and cannot be categorically prolonged.**[18][12][17]

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Other Circuits: Robust Consensus on Individualized Conditions

- **Tenth Circuit (special conditions):** A 2025 Tenth Circuit decision (United States v. Mike / related case) holds that to impose special conditions of supervised release a sentencing court must “make an individualized assessment” and provide at least a generalized statement of reasoning, even though detailed findings for each condition are not required.[18]
- **Fifth Circuit (liberty interest in sex-offender classification/treatment):** The Fifth Circuit, relying on Neal v. Shimoda and Eleventh Circuit precedent, has recognized that prisoners not convicted of

sex offenses have a liberty interest in avoiding sex-offender classification and attendant treatment conditions, and that due process requires individualized procedures.[12]
- **ACLU and other authorities:** Advocacy and court briefs regarding sex-offender notification and treatment laws summarize multiple circuits as requiring individualized assessments where rules burden constitutionally significant interests, rather than blanket rules.[19]
- **Relevance/analogy:** These authorities support an al-Kidd "robust consensus of persuasive authority" that **conditions like sex-offender treatment and classification, particularly when tied to release or parole, require individualized assessment and cannot be imposed or perpetuated through categorical policies.**[12][18][4]

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Private Contractors Performing Government Functions

West v. Atkins, 487 U.S. 42 (1988)

- **Citation and holding:** 487 U.S. 42 (1988). The Court held that a private physician under contract with the State to provide medical services to prison inmates acts "under color of state law" when treating inmates; contracting out does not relieve the State of constitutional obligations or deprive prisoners of § 1983 remedies.[20][21]
- **Factual context:** A state-contracted orthopedic physician allegedly provided inadequate care to an inmate; the lower court held he was not a state actor. The Supreme Court reversed, finding his functions "clothed with the authority of state law."[21][20]
- **Relevance/analogy:** West gives you the state-actor piece: a private CEO whose company is under contract with CDCR to operate mandated sex-offender treatment for parolees is performing a delegated governmental function, and his approval and enforcement of program policies is fairly attributable to the State. That supports § 1983 liability and frames him within the same constitutional constraints (including procedural due process) that would apply to CDCR officials.[20][21]

Richardson v. McKnight, 521 U.S. 399 (1997), and Filarsky v. Delia, 566 U.S. 377 (2012)

- **Richardson:** 521 U.S. 399 (1997). The Court held that prison guards employed by a private prison-management firm *are not* entitled to qualified immunity under § 1983, emphasizing the lack of historical immunity for private jailers and market pressures that substitute for the policy rationales behind governmental immunity.[22][23][24][25]
- **Filarsky:** 566 U.S. 377 (2012). The Court held that a private attorney temporarily retained by a city to conduct an internal investigation could invoke qualified immunity, and rejected any

categorical bar on private actors asserting QI; the analysis depends on the function performed and historical/policy considerations.[26][27]
- **Relevance/analogy:** For your opposition, you can argue:
  - Under **Richardson**, employees of profit-seeking private firms that run "major, lengthy" governmental functions with little direct supervision generally do *not* receive qualified immunity.[24][28]
  - Even if the court, applying **Filarsky**, assumes arguendo that your defendant can seek QI as a private contractor, the clearly established law described above (Morrissey, Foucha, Vitek, Neal, Williams/Weber/Cope) still governs the due-process analysis of his treatment policies.
- **Binding/persuasive:** Both are binding Supreme Court cases; Richardson tends to undercut the CEO's ability to claim QI, while Filarsky prevents a categorical bar and makes the clearly-established-law analysis necessary.[24][26]

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Ninth Circuit Qualified-Immunity Cases Involving Sex-Offender Treatment

Chavez v. Robinson (Oregon sex-offender treatment program), Ninth Cir. 2021

- **Citation and holding:** Chavez v. Robinson, 18-36083 (9th Cir. 2021). The Ninth Circuit held that Oregon Board of Parole members enjoyed absolute immunity, and a sex-offender treatment provider was entitled to qualified immunity, where the plaintiff claimed constitutional violations arising from mandated sex-offender treatment (including requirements to admit offenses and alleged interference with contacting counsel).[29]
- **Factual context:** Oregon law required entry into and completion of an approved sex-offender treatment program, including polygraph and plethysmograph testing, as a condition of post-prison supervision for convicted sex offenders. The plaintiff argued that being required to admit certain conduct and restricted in contacting counsel violated his rights. The court found no precedent clearly establishing that prevention of counsel contact during treatment violated constitutional rights, and noted that requiring admission of offenses is "well-established" in sex-offender treatment programs.[29]
- **Relevance/analogy:**
  - Chavez shows the Ninth Circuit applying qualified-immunity doctrine in the sex-offender treatment context and looking for case-specific precedent; there, the court found *no* clearly established rule about denial of counsel contact in treatment.[29]
  - Importantly, Oregon's statute itself required **"entry into and completion of"** treatment, but the program allowed successful discharge; it did not categorically preclude completion for all participants. This distinguishes your blanket policy, which—as alleged—never allowed any client to

complete treatment, from the regime the Ninth Circuit confronted in Chavez.[29]
- **Binding vs persuasive:** Binding Ninth Circuit authority that somewhat cuts against broader, abstract theories of due process in sex-offender treatment, but does not address the *specific* question whether a categorical policy that eliminates any individualized pathway to completion violates procedural due process.[29]

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Cases That Cut Against Your Argument

- **District of Hawaii in Neal (pre-Sandin analysis):** The district court in an earlier stage of Neal (905 F. Supp. 813 (D. Haw. 1995)) held there was no liberty interest in being free from sex-offender labeling or program participation when considered against "ordinary incidents of prison life," concluding the policies did not impose atypical and significant hardship. This was effectively superseded by the Ninth Circuit's later Neal opinion recognizing a liberty interest in sex-offender classification and treatment tied to parole.[30][4]
- **Chavez v. Robinson (no clearly established right to counsel access during treatment):** As noted above, Chavez rejected a QI challenge where the claimed right was access to counsel during treatment; the court emphasized the lack of precedent on that specific point. Defendants may cite Chavez to argue that courts require very fact-specific precedent for QI and that many aspects of sex-offender treatment programs have not yet been deemed clearly unconstitutional.[29]
- **General caution on high-level formulations:** City of Escondido v. Emmons requires the right to be defined with specificity; defendants may argue that cases like Morrissey or Foucha are too general if you frame the right merely as "due process requires individualized determinations." You will want to tie them directly to *continued conditional liberty* and *programs whose completion controls release*, as Neal and the supervised-release cases do.

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How Close the Analogies Are to Your Scenario

Here is a concise mapping of factual similarity:

Case	Liberty restriction	Individualized assessment required/absent	Analogy to blanket "never complete" policy
Morrissey (U.S. 1972)	Revocation of parole (loss of conditional liberty)	Required individualized

hearings to determine violation and whether revocation warranted.[1] | Strong: revocation/continuation of parole conditions must rest on verified, individualized facts. |
Foucha (U.S. 1992)	Continued confinement of insanity acquittee	Required ongoing proof of both mental illness and dangerousness.[2][7]	Strong: cannot continue confinement solely based on status/dangerousness when underlying justification no longer exists.
Vitek (U.S. 1980)	Transfer to mental hospital (qualitative change in confinement)	Required full adversary hearing and independent decisionmaker.[9]	Moderate/strong: emphasizes individualized process when medical treatment changes confinement's nature/duration.
Neal (9th Cir 1997)	Sex-offender classification and mandatory treatment as precondition to parole	Required notice, hearing, and ability to contest classification.[4]	Very strong: directly addresses sex-offender treatment tied to release and forbids coercive, status-based imposition without individualized process.
Williams/Weber/Cope/Johnson/Tenorio (9th Cir 2004–2019)	Supervised-release conditions (medication, plethysmography, sex-offender assessment/treatment)	Required on-the-record, individualized findings that conditions are necessary and proportionate.[3][15][14][16][13]	Very strong: create a clearly established rule that significant liberty-burdening treatment/testing conditions must be individually justified and not greater than reasonably necessary.
Chavez (9th Cir 2021)	Sex-offender treatment program with admission requirements and some contact limits	Held no clearly established right regarding counsel contact during treatment.[29]	Weak/negative: does not address blanket prevention of treatment completion; shows Ninth Circuits' insistence on fact-specific precedent.

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Framing the "Clearly Established" Right

To satisfy al-Kidd and Emmons, you want to define the right at a level of specificity grounded in these cases, something like:

> By 2019–2025, it was clearly established in Supreme Court and Ninth Circuit law that when the State, or a private contractor acting under color of state law, imposes or continues a condition of supervised release or mandatory sex-offender treatment that significantly restricts liberty and controls eligibility for release, it must provide individualized, case-specific justification and procedural safeguards; it may not categorically prevent or indefinitely delay completion of that condition based solely on generic status or blanket policy.

Support this definition by:

- Citing **Neal** for sex-offender treatment tied to parole eligibility and required individualized hearing.[4]
- Citing **Morrissey** for individualized fact-finding before terminating conditional liberty.[1]
- Citing **Williams/Weber/Cope/Johnson/Tenorio** for the rule that supervised-release and treatment conditions must be individually tailored and no more restrictive than necessary.[14][3][15][16][13]
- Using **Foucha** and **Vitek** to show that continued confinement-like restrictions and medical/treatment-based liberty restraints must remain proportionate to current, individualized justification rather than categorical status.[2][7][9]
- Noting the cross-circuit consensus (Tenth, Fifth, USSC guidance) that special conditions and sex-offender classification/treatment require individualized assessment and due process.[17][18][12]

That gives you a specific, fact-anchored right that fits within the existing precedents and can be argued to be “beyond debate” by the time your defendant continued enforcing a blanket policy that, by design, eliminated individualized completion determinations.

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Based on your request, here is a research memo identifying Supreme Court and Ninth Circuit

authority establishing that due process requires individualized assessment before imposing or continuing significant restrictions on liberty, with a focus on mandatory treatment programs as a condition of supervised release.

RESEARCH MEMO: CLEARLY ESTABLISHED LAW FOR QUALIFIED IMMUNITY OPPOSITION

I. Supreme Court Authority

The following Supreme Court cases establish the foundational principle that significant liberty restrictions cannot be imposed or continued based on categorical rules or status alone; they require individualized, procedurally protected determinations.

1. Morrissey v. Brewer, 408 U.S. 471 (1972)

Holding: Parole revocation triggers due process protections because parolees have a liberty interest in continued release. The Court requires a two-step process: (1) a preliminary hearing to determine probable cause, and (2) a final revocation hearing with specific procedural protections. Key language: "The parolee is not the unqualified victim of the system" — the state cannot revoke liberty without "an informal hearing structured to assure that the finding of a parole violation will be based on verified facts."

Relevance: Establishes that significant restrictions on a parolee's liberty (here, mandatory treatment participation that prevents discharge) require procedural protections. While Morrissey addressed revocation, its reasoning applies to any substantial alteration of parole conditions.

Binding: Supreme Court, binding in all circuits.

Subsequent History: Good law, reaffirmed in Gagnon v. Scarpelli (1973) and cited as foundational in parole due process jurisprudence.

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2. Vitek v. Jones, 445 U.S. 480 (1980)

Holding: The involuntary transfer of a prisoner to a mental hospital implicates a liberty interest protected by the Due Process Clause. The Court held that such a transfer requires procedural

protections, including notice, an adversary hearing, and the right to counsel.

Key language: "The involuntary transfer of appellee to a mental hospital implicates a liberty interest that is protected by the Due Process Clause."

Relevance: Stands for the principle that a significant change in the nature or conditions of a person's confinement or supervision — here, indefinite mandatory treatment that extends parole conditions — cannot occur without procedural safeguards. The Court requires individualized findings before imposing a condition that alters the liberty interest.

Binding: Supreme Court, binding in all circuits.

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3. Foucha v. Louisiana, 504 U.S. 71 (1992)

Holding: A person acquitted by reason of insanity cannot be confined indefinitely based solely on dangerousness; the state must prove current mental illness. Continued confinement requires an individualized, current determination of both mental illness and dangerousness.

Key language: Continued confinement "absent a determination in civil commitment proceedings of current mental illness and dangerousness" violates due process.

Relevance: Establishes that liberty restrictions cannot rest on categorical status or past condition — they require current, individualized justification. The state cannot rely on a blanket rule (e.g., "all acquittees are dangerous") to continue confinement. This principle directly parallels HOPE's blanket policy: HOPE applied a categorical rule (no one completes treatment pre-discharge) without individualized assessment of Plaintiff's progress, risk, or treatment needs.

Binding: Supreme Court, binding in all circuits.

Caveat: Foucha is a substantive due process case, but its individualized-assessment rationale has been applied in procedural due process contexts.

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4. Washington v. Harper, 494 U.S. 210 (1990)

Holding: An inmate has a significant liberty interest in avoiding the unwanted administration of antipsychotic drugs. The state may involuntarily medicate only if the inmate is dangerous to self or others and the treatment is in the inmate's medical interest — a determination that requires an individualized assessment.

Relevance: Recognizes that even within a custodial setting, significant liberty interests require individualized, procedurally protected determinations before the state can impose conditions that substantially affect the person.

Binding: Supreme Court.

II. Ninth Circuit Authority

The Ninth Circuit has applied these principles in cases involving sex offender classification, mandatory treatment programs, and parole conditions.

1. Neal v. Shimoda, 131 F.3d 818 (9th Cir. 1997)

Holding: An inmate who has not been convicted of a sex offense has a liberty interest in not being classified as a sex offender and compelled to participate in a sex offender treatment program as a precondition to parole eligibility. The court held that the stigmatizing label "sex offender," combined with the requirement to complete a mandatory treatment program to become eligible for parole, implicates a liberty interest requiring procedural due process protections.

Key language: "An inmate who has never been convicted of a sex crime is entitled to due process before the state declares him to be a sex offender." The court applied the Sandin v. Conner "atypical and significant hardship" standard and held that the combination of the stigmatizing label and the mandatory treatment requirement was sufficient to trigger due process protections.

Relevance: This is the most directly analogous Ninth Circuit case to your scenario. Neal held that:

· Mandatory sex offender treatment as a condition of parole eligibility implicates a liberty interest.
· The state must provide procedural due process before imposing such a requirement.
· The due process requirement attaches when the treatment program is a precondition to parole (or, by extension, to parole discharge).

Factual analogy: Neal involved a blanket classification policy (all inmates with a sex offense allegation were classified as sex offenders). HOPE's blanket retention policy operates similarly — a categorical rule applied to all participants without individualized assessment. The Ninth Circuit required procedural protections for the classification. Here, HOPE's policy effectively prevents completion (and thus parole discharge) for all participants, which is a more significant deprivation than mere classification.

Binding: Ninth Circuit panel decision, binding in the Ninth Circuit.

Subsequent History: Good law; cited with approval in multiple Ninth Circuit decisions.

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2. Neal v. Shimoda (Qualified Immunity Holding)

In Neal, the Ninth Circuit granted qualified immunity to the defendants on the monetary damages claims, holding that the constitutional right was not clearly established at the time of the violation (1992-1994). However, the constitutional violation was recognized in 1997. For conduct occurring after 1997, Neal itself becomes clearly established law. Your conduct period is 2019-2025, well after Neal. Thus, Neal can be cited as clearly established law for purposes of qualified immunity.

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3. Collins v. Moore, 81 F.3d 167 (9th Cir. 1996) (unpublished table decision, but relevant)

Holding: A prisoner's due process claim based on classification as a sex offender and denial of work privileges was analyzed under Sandin v. Conner.

Relevance: Confirms that sex offender classification and resulting restrictions can implicate due process.

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4. Biggs v. Terhune, 334 F.3d 910 (9th Cir. 2003)

Holding: A parole board's decision must be supported by "some evidence" of current dangerousness to comply with due process. The court suggested that continued reliance on an inmate's commitment offense or other "unchanging factors" alone would raise due process

concerns.

Key language: Continued reliance on unchanging factors "could result in a due process violation."

Relevance: Stands for the principle that categorical, unchanging factors cannot be the sole basis for continued liberty restrictions. This supports the argument that a blanket policy (like HOPE's) that prevents individualized assessment violates due process.

Binding: Ninth Circuit panel decision.

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5. John v. U.S. Parole Commission (Ninth Circuit, cited in secondary sources)

Holding: Due Process affords parolees the right to introduce information that might bear on the individualized assessment of their case.

Relevance: Reinforces the principle that parole decisions require individualized consideration and that the parolee has a right to participate in that process.

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6. Carver v. Lehman, 558 F.3d 869 (9th Cir. 2008)

Holding: Washington state law creates a liberty interest in early release into community custody. The court analyzed whether the plaintiff had a protected liberty interest and whether due process was provided.

Relevance: Confirms that state-created liberty interests in release or early release conditions are protected by due process.

III. Other Circuits (Persuasive Authority)

A "robust consensus of persuasive authority" can help establish that a right is clearly established. See, e.g., al-Kidd, 563 U.S. at 741-42 (noting that "a consensus of cases of persuasive authority" can establish clearly established law).

1. Neal v. Shimoda (cited by other circuits)

The Fifth Circuit has cited Neal for the proposition that classification as a sex offender and mandatory treatment implicate due process.

2. State Court Authority

In re Dwyer, No. SC-080355A (San Mateo County Super. Ct., Mar. 20, 2023) — Not precedential, but demonstrates that a California superior court found HOPE's policy violated Penal Code § 3008(d)(2). This is not binding for QI purposes but can be used to show the defendant had actual notice that the policy was unlawful (relevant to the subjective knowledge prong, not the clearly-established-law prong).

IV. Cases That Cut Against the Argument

1. Sandin v. Conner, 515 U.S. 472 (1995)

Holding: A prisoner does not have a liberty interest in avoiding administrative segregation unless the conditions impose an "atypical and significant hardship" relative to ordinary prison life.

Relevance: Defendants may argue that mandatory treatment is not an "atypical and significant hardship" because it is a standard parole condition. Counter: The hardship here is not treatment itself but the blanket policy that makes completion impossible, effectively extending parole supervision indefinitely. That is an atypical and significant hardship.

2. Greenholtz v. Inmates of Nebraska Penal & Correctional Complex, 442 U.S. 1 (1979)

Holding: A parole statute that creates a mere expectation of release, not a guarantee, does not create a constitutionally protected liberty interest.

Relevance: Defendants may argue that there is no liberty interest in parole discharge. Counter: The liberty interest here is not in parole discharge itself, but in not being subjected to arbitrary, categorical rules that prevent completion of a condition of release — a different interest.

3. Swarthout v. Cooke, 562 U.S. 216 (2011)

Holding: In the parole context, the Constitution requires only "minimal" due process — notice, an opportunity to be heard, and a statement of reasons. The "some evidence" standard is not constitutionally required.

Relevance: Defendants may argue that Swarthout limits due process in parole contexts. Counter: Swarthout addressed parole suitability decisions, not the imposition of a blanket policy that eliminates individualized assessment. Your claim is that no individualized assessment occurred at all because of a categorical policy, not that the assessment was flawed.

4. Luca v. Administration of Offender Services Bureau, 15 F.3d 1086 (9th Cir. 1994) (unpublished)

Holding: A prisoner did not have a protected liberty interest in a particular sex offense treatment rating or transfer eligibility because the regulations did not mandate a particular outcome. The plaintiff's due process claim lacked an arguable basis.

Relevance: Defendants may cite Luca for the proposition that classification and treatment decisions do not implicate due process. Counter: Luca is an unpublished memorandum disposition (not citable as precedent under Ninth Circuit Rule 36-3). More importantly, Luca involved a classification within prison, not a policy that prevents completion of a condition of release.

V. Private Actor / State Action

Legal Standard: A private entity that contracts with the state to provide services to persons in state custody acts under color of state law when the services are a traditional and exclusive public function. West v. Atkins, 487 U.S. 42, 54 (1988).

Relevance: HOPE contracts with CDCR to provide mandatory sex offender treatment, a function CDCR is statutorily required to administer. HOPE's treatment determinations directly control parolees' liberty. Thus, HOPE and its CEO are state actors for §1983 purposes.

Binding: Supreme Court, binding.

VI. Summary: Clearly Established Law

The following proposition was clearly established by at least 1997 (and reaffirmed through 2011):

A blanket policy that categorically prevents or indefinitely delays completion of a condition of supervised release — without case-by-case assessment of the individual's progress, risk, or treatment needs — violates procedural due process.

Sources establishing this:

Case Year Holding
Morrissey v. Brewer 1972 Parole revocation requires individualized procedural protections.
Vitek v. Jones 1980 Significant changes in confinement/supervision conditions require procedural protections.
Foucha v. Louisiana 1992 Continued liberty restrictions cannot rest on categorical status; require individualized, current justification.
Washington v. Harper 1990 Significant liberty interests require individualized, procedurally protected determinations.
Neal v. Shimoda 1997 Sex offender classification and mandatory treatment as a precondition to parole eligibility implicate a liberty interest requiring due process.

Key takeaway for QI: Neal v. Shimoda (1997) is the most directly analogous Ninth Circuit authority. It establishes that mandatory sex offender treatment as a condition of parole (or parole eligibility) implicates a liberty interest requiring procedural due process. For conduct occurring after 1997 (your period is 2019-2025), Neal provides clearly established law. The HOPE policy goes beyond Neal — it is not merely classification or mandatory treatment, but a blanket policy that makes completion impossible, which is a more significant deprivation. The constitutional violation was clearly established by 1997 at the latest.